

This is a combined synopsis/solicitation for commercial items prepared in accordance with the format in Federal Acquisition Requirements (FAR) Subpart 12.6, as supplemented with additional information included in this notice. This announcement constitutes the only solicitation; proposals are being requested, and a written solicitation will not be issued.

The solicitation number for this requirement is **75D301-26-Q-97398** and is hereby issued as a Request for Quote (RFQ) using FAR Subpart 13.1 Simplified Acquisition Procedures.

Item	Supplies/Services	Qty/Unit	Unit Price	Total
0001	Equipment Services This is a Firm fixed price line item This is a Non-severable line item Period of Performance is on or before June 30, 2026	_____	\$ _____	\$ _____

(All work shall be performed in accordance with the attached Statement of Work (SOW))

STATEMENT OF WORK

Period of performance of this contract is: June 30, 2025

SECTION 1 – BACKGROUND

The National Personal Protective Technology Laboratory (NPPTL), National Institute for Occupational Safety and Health (NIOSH), and Center for Disease Control and Prevention (CDC) conducts respirator approval testing. This includes approval and audit testing for CBRN, SCBA, CCER, and Firefighting respirators. These respirators are each subjected to extreme environmental conditions, which are reproduced as part of the NIOSH Standard Testing Procedures. The laboratory owns and operates a set of four environmental chambers which are able to produce a wide range of environmental conditions. This contract is for the preventative maintenance and ISO/IEC 17025 accredited calibration of the environmental chambers which are required to perform the NIOSH respirator approval and audit testing.

SECTION 2 – PURPOSE/OBJECTIVE

The purpose of this contract is for the preventative maintenance and ISO/IEC 17025 accredited calibration of four environmental chambers located on the NIOSH Pittsburgh campus. The equipment requires basic maintenance and regular calibration.

SECTION 3 – SCOPE OF WORK

The scope of this contract is limited to the four Russells Technical Products environmental chamber in B40 on the NIOSH Pittsburgh campus. The scope will include the preventative maintenance and ISO/IEC 17025 accredited calibration of each chamber.

SECTION 4 – TASKS TO BE PERFORMED

Tasks will be performed on the following equipment:

Model: WMD-1407-20-HCD-1125-5-AC, Serial # 07164668

Model: WMD-288-GCMD-6-6-AC, Serial # 04184853

Model: WMD-900-30-30-MUA-AC, Serial # 07174794

Model: WMD-900-30-30-MUA-60-AC, Serial # 05174750

Task 1: Standard preventative maintenance, per manufacturer's specifications and guidance.

Task 2: ISO/IEC 17025 accredited calibration. Calibration Test Points: Temperature: -50.0 °C, -17.5 °C, 15.0 °C, 47.5 °C, 80.0°C. Humidity: + 40.0 °C @ 0.0 %RH, 22.5 %RH, 45.0 %RH, 67.5 %RH, 90.0 %RH.

SECTION 5 – GOVERNMENT FURNISHED PROPERTY

The government shall provide a secure, temperature controlled laboratory facility to work in. Electrical connections, compressed air, and municipal water connections are available for use. A small supply of spare and consumable parts is available, but inventory is unknown and should not be relied upon. Basic access to restroom and break room areas will be allowed.

SECTION 6 – PLACE OF PERFORMANCE AND CONTRACT SUPPORT HOURS

SUBSECTION A – PLACE OF PERFORMANCE

CDC/NIOSH
 Bruceton Campus
 Building 40
 626 Cochrans Mill Rd.
 Pittsburgh, PA 15236

SUBSECTION B – CONTRACT SUPPORT HOURS

Vendor shall coordinate with government technician prior to scheduling travel. Typical working hours for the purpose of this contract are 9am to 3:30pm.

SECTION 7 – TRAVEL

The contract shall include all costs related to vendor travel to the NIOSH location at the address listed above. There is no travel for government personnel.

SECTION 8 – DELIVERABLES/REPORTING SCHEDULE

The preventative maintenance and calibration should be performed within six months of the date of contract award. Reporting of calibration results and 17025 calibration certificate shall be delivered within one week of the completed calibration.

SECTION 9 – REFERENCE MATERIALS

None.

SECTION 10 – MINIMUM VENDOR QUALIFICATIONS

Contract is required with the original equipment manufacturer, Russells Technical Products, Inc., or their subcontractor.

SECTION 11 – ADDITIONAL REQUIREMENTS

None.

Section 508 and Accessibility Requirements**Part A – Provision**

Reference HHSAR 352.239-73[8] Electronic Information and [Communication] Technology Accessibility Notice.

[(Deviation)]

[(a) Any offeror responding to this solicitation must comply with established HHS Information and Communication Technology (ICT) accessibility standards. Information about Section 508 is available at <https://www.hhs.gov/web/section-508/index.html>.

(b) The Section 508 accessibility standards applicable to this solicitation are stated in the clause at 352.239-79 Information and Communication Technology Accessibility. In order to facilitate the Government's determination whether proposed ICT supplies, products, platforms, information, and documentation meet

applicable Section 508 accessibility standards, offerors must submit an appropriate HHS Section 508 Accessibility Conformance Checklist (see <https://www.hhs.gov/web/section-508/accessibility-checklists/index.html>) or an Accessibility Conformance Report (ACR) (based on the Voluntary Product Accessibility Template (VPAT) see <https://www.itic.org/policy/accessibility/vpat>), in accordance with the completion instructions. The purpose of the checklists and conformance reports are to assist HHS acquisition and program officials in determining whether proposed ICT supplies, products, platforms, information, and documentation conform to applicable Section 508 accessibility standards. Checklists and ACRs evaluate—in detail—whether the ICT conforms to specific Section 508 accessibility standards and identifies remediation efforts needed to address conformance issues.

(c) If an offeror claims its supplies or services meet applicable Section 508 accessibility standards, and it is later determined by the Government, *i.e.*, after award of a contract or order, that supplies, products, platforms, information, documentation, or services support delivered do not conform to the described accessibility standards, remediation of the supplies, products, platforms, information, documentation, or services support to the level of conformance specified in the contract will be the responsibility of the Contractor at its expense.

(d) In order to facilitate the Government's determination whether proposed ICT supplies meet applicable Section 508 accessibility standards, offerors must submit an Accessibility Conformance Report, in accordance with its completion instructions and tailored to the requirements in the solicitation. The purpose of the Report is to assist HHS acquisition and program officials in determining whether proposed ICT supplies conform to applicable Section 508 accessibility standards. The template allows offerors or developers to self-evaluate their supplies and document, in detail, whether they conform to a specific Section 508 accessibility standard, and any underway remediation efforts addressing conformance issues. Instructions for preparing the Template are available at <https://www.section508.gov/>.

(e) Additionally, offerors must provide enough information to assist the Government in determining that the ICT services conform to Section 508 accessibility standards, including any underway remediation efforts addressing conformance issues.

(f) Respondents to this solicitation must identify any inability to conform to Section 508 requirements. If an offeror claims its supplies or services meet applicable Section 508 accessibility standards, and it is later determined by the Government, *i.e.*, after award of a contract or order, that supplies or services delivered do not conform to the described accessibility standards, remediation of the supplies or services to the level of conformance specified in the contract will be the responsibility of the Contractor at its expense.

(g) Items delivered as electronic content must be accessible to HHS acceptance criteria. Checklist for various formats are available at <http://508.hhs.gov/>. Materials, other than items incidental to contract management, that are final items for delivery should be accompanied by the appropriate checklist, except upon approval of the Contracting Officer or Contracting Officer's Representative.

(End of provision)]

Part B – Clause

HHSAR 352.239-74[9] Electronic, Information, and Communication Technology Accessibility.

[(Deviation)]

(a) Pursuant to Section 508 of the Rehabilitation Act of 1973 (29 U.S.C. 794d), as amended by the Workforce Investment Act of 1998, all information and communication technology (ICT) supplies, products, platforms, information, documentation, and services, or support developed, acquired, maintained or delivered under this contract or order must comply with the Revised 508 Standards, which are located at 36 C.F.R. 1194.1 and Appendices A, B, and C, and are available at <https://www.access-board.gov/ict/>. Information about Section 508 is available at <https://www.hhs.gov/web/section-508/index.html>.

(b) Additional Section 508 accessibility standards applicable to this contract or order are identified as follows:

205 WCAG 2.0 Level A & AA Success Criteria
 302 Functional Performance Criteria
 502 Inoperability with Assistive Technology
 504 Authoring Tools
 602 Support Documentation
 603 Support Services

If it is determined by the Government that ICT supplies, products, platforms, information, documentation, and services support provided by the Contractor do not conform to the described accessibility standards in the contract, remediation of the supplies, products, platforms, information, documentation, or services support to the level of conformance specified in the contract will be the responsibility of the Contractor at its own expense.

(c) In the event of a modification(s) to this contract or order, which adds new ICT supplies or services or revises the type of, or specifications for, supplies, products, platforms, information, documentation, or services support, the Contracting Officer shall require that the Contractor submit a completed HHS Section 508 Accessibility Conformance Checklist (see <https://www.hhs.gov/web/section-508/accessibility-checklists/index.html>) or an Accessibility Conformance Report (ACR) (based on the Voluntary Product Accessibility Template (VPAT) see <https://www.itic.org/policy/accessibility/vpat>), and any other additional information necessary to assist the Government in determining that the ICT supplies or services conform to Section 508 accessibility standards. If it is determined by the Government that ICT supplies, products, platforms, information, documentation, and services support provided by the Contractor do not conform to the described accessibility standards in the contract, remediation of the supplies, products, platforms, information, documentation, or services support to the level of conformance specified in the contract will be the responsibility of the Contractor at its own expense.

(d) If this is an Indefinite-Delivery type contract, a Blanket Purchase Agreement or a Basic Ordering Agreement, the task/delivery order requests that include ICT supplies, products, platforms, information, documentation, or services support will define the specifications and accessibility standards for the order. In those cases, the Contractor shall be required to provide a completed HHS Section 508 Accessibility Conformance Checklist (see <https://www.hhs.gov/web/section-508/accessibility-checklists/index.html>) or an ACR (based on the VPAT see <https://www.itic.org/policy/accessibility/vpat>), and any other additional information necessary to assist the Government in determining that the ICT supplies, products, platforms, information, documentation, or services support conform to Section 508 accessibility standards. If it is determined by the Government that ICT supplies and services provided by the Contractor do not conform to the described accessibility standards in the provided documentation, remediation of the supplies, products, platforms, information, documentation, or services support to the level of conformance specified in the contract will be the responsibility of the Contractor at its own expense.

(e) The contractor shall identify to the Contracting Officer any perceived exception or exemption to Section 508 requirements for review by the agency Section 508 Program.

CONTRACT TERMS AND CONDITIONS

Clauses By Reference

FAR SOURCE	TITLE AND DATE
52.203-17	Contractor Employee Whistleblower Rights (2023)
52.203-19	Prohibition on Requiring Certain Internal Confidentiality Agreements or Statements (Jan 2017)
52.204-13	52.204-13 System for Award Management Maintenance (DEVIATION) (RFO AUG 2025)
52.209-6	Protecting the Government's Interest When Subcontracting With Contractors Debarred, Suspended, Proposed for Debarment, or Voluntarily Excluded (DEVIATION) (RFO AUG 2025)
52.209-10	Prohibition on Contracting with Inverted Domestic Corporations (DEVIATION) (RFO AUG 2025)
52.212-4	Contract Terms and Conditions—Commercial Products and Commercial Services (DEVIATION) (RFO AUG 2025)
52.219-6	Notice of Total Small Business Set-Aside (Nov 2020)
52.222-3	Convict Labor (JUN 2003)
52.222-19	Child Labor-Cooperation with Authorities and Remedies (JAN 2025)
52.222-36	Equal Opportunity for Workers with Disabilities (JUN 2020)
52.222-50	Combating Trafficking in Persons (NOV 2021)
52.223-23	Sustainable Products and Services (MAY 2024) (DEVIATION FEB 2025)
52.226-8	Encouraging Contractor Policies to Ban Text Messaging While Driving (MAY 2024)
52.232-33	Payment by Electronic Funds Transfer-System for Award Management (OCT 2018)
52.232-40	Providing Accelerated Payments to Small Business Subcontractors (Mar 2023)
52.233-3	Protest after Award (DEVIATION) (RFO AUG 2025)
52.233-4	Applicable Law for Breach of Contract Claim (DEVIATION) (RFO AUG 2025)
52.240-91	Security Prohibitions and Exclusions (DEVIATION) (RFO AUG 2025)
52.244-6	Subcontracts for Commercial Products and Commercial Services (Jan 2025) (DEVIATION) (RFO SEP 2025)

Clauses In Full Text

HHSAR 352.204-71 Information and Information Systems Security. (Deviation)(Feb 2024)

(a) Definitions. As used in this clause—

Breach means the loss of control, compromise, unauthorized disclosure, unauthorized

acquisition, or any similar occurrence where—

- (1) A person other than an authorized user accesses or potentially accesses personally identifiable information, or
- (2) An authorized user accesses personally identifiable information for an other than authorized purpose.

Business associate (see 45 CFR 160.103), except as provided in paragraph (2) of this definition, business associate means, with respect to a covered entity, a person who -

(1) On behalf of such covered entity or of an organized health care arrangement (as defined in this clause) in which the covered entity participates, but other than in the capacity of a member of the workforce of such covered entity or arrangement, creates, receives, maintains, or transmits protected health information for a function or activity regulated by this contract or agreement, including claims processing or administration, data analysis, processing or administration, utilization review, quality assurance, patient safety activities listed at 42 CFR 3.20, billing, benefit management, practice management, and repricing; or

(2) Provides, other than in the capacity of a member of the workforce of such covered entity, legal, actuarial, accounting, consulting, data aggregation (as defined in 45 CFR section 164.501), management, administrative, accreditation, or financial services to or for such covered entity, or to or for an organized health care arrangement in which the covered entity participates, where the provision of the service involves the disclosure of protected health information from such covered entity or arrangement, or from another business associate of such covered entity or arrangement, to the person.

(3) A covered entity may be a business associate of another covered entity.

(4) Business associate includes the following:

(i) A Health Information Organization, E-prescribing Gateway, or other person that provides data transmission services with respect to protected health information to a covered entity and that requires access on a routine basis to such protected health information.

(ii) A person that offers a personal health record to one or more individuals on behalf of a covered entity.

(iii) A subcontractor that creates, receives, maintains, or transmits protected health information on behalf of the business associate.

(5) Business associate does not include:

(i) A health care provider, with respect to disclosures by a covered entity to the health care provider concerning the treatment of the individual.

(ii) A plan sponsor, with respect to disclosures by a group health plan (or by a health insurance issuer or HMO with respect to a group health plan) to the plan sponsor, to the extent that the requirements of 45 CFR 164.504(f) apply and are met.

A government agency, with respect to determining eligibility for, or enrollment in, a government health plan that provides public benefits and is administered by another government agency, or collecting protected health information for such purposes, to the extent such activities are authorized by law.

(iii) A covered entity participating in an organized health care arrangement that performs a function or activity as described by paragraph (1)(i) of this definition for or on behalf of such organized health care arrangement, or that provides a service as described in paragraph (1)(ii) of this definition to or for such organized health care arrangement by virtue of such activities or services.

Business associate agreement means the agreement, or other arrangement, as dictated by the HIPAA Privacy Rule (45 CFR 160), between an HHS covered entity and a business associate, which must be entered into in addition to the underlying contract for services and before any disclosure (see 45 CFR 160.103) of PHI can be made to the business associate, in order for the business associate to perform certain functions or activities on behalf of an HHS entity.

Controlled unclassified information (CUI) means information that laws, regulations, or Government-wide policies require to have safeguarding or dissemination controls, excluding classified information.

Healthcare component means a component or combination of components of a hybrid entity designated by the hybrid entity in accordance with 45 CFR 164.105(a)(2)(iii)(D) (see 45 CFR 164.103). The Secretary of HHS has designated HHS as a covered entity (further designated as a “hybrid entity”), and has also designated four HHS divisions as healthcare components under HIPAA, including —

- (1) The Centers for Medicare and Medicaid Services (CMS), insofar as it operates the fee-for-service Medicare program;
- (2) The Program Support Center (PSC), Division of Commissioned Personnel, insofar as it operates a health plan for Commissioned Corps officers;
- (3) The World Trade Center (WTC) Health Program; and,
- (4) The Indian Health Service (IHS), insofar as it operates a health plan and a program providing healthcare that uses electronic transactions.

HHS Information Technology General Rules of Behavior means a set of HHS rules that describes the responsibilities and expected behavior of users of HHS information or information systems.

HHS sensitive information means all HHS data, on any storage media or in any form or format, which requires confidentiality, integrity, and availability protection due to the risk of harm that could result to interests of HHS, other agencies or entities, or individuals from inadvertent or deliberate disclosure, alteration, or destruction of the information. The term includes—

- (1) Information where the improper use or disclosure could adversely affect the ability of HHS to accomplish its mission, i.e., HHS proprietary information;
- (2) Records about individuals requiring protection under laws and regulations such as the E-Government Act, Privacy Act, and the HIPAA Privacy Rule, or based on a data use agreement or a promise or assurance of confidentiality; and
- (3) Information that would be exempt from disclosure if requested under the Freedom of Information Act. Examples of HHS sensitive information include—
 - (i) Individually-identifiable medical, benefits, and personnel information;
 - (ii) Financial, budgetary, research, quality assurance, confidential commercial, critical infrastructure, security-sensitive, procurement-sensitive, investigatory, and law enforcement information; Controlled unclassified information;
 - (iii) Information that would be confidential and privileged in litigation such as information protected by the deliberative process privilege, attorney work-product privilege, and the attorney-client privilege; and
 - (iv) Other information which, if released, could result in a violation of law or agreement, could cause harm or unfairness to any individual or group, or could adversely affect the national interest or the conduct of Federal programs.

HIPAA Rules means the Privacy, Security, Breach Notification, and Enforcement Rules at 45 CFR Part 160 and part 164.

Incident means an occurrence that (A) actually or imminently jeopardizes, without lawful authority, the integrity, confidentiality, or availability of information systems; or (B) constitutes a violation or imminent threat of violation of law, security policies, security procedures, or acceptable policies.

Information system means a discrete set of information resources organized for the collection, processing, maintenance, use, sharing, dissemination, or disposition of information.

Information system security plan means a formal document that provides an overview of the security requirements for an information system or an information security program and describes the security controls in place or planned for meeting those requirements.

Information technology (see FAR 2.101) also means Information and Communication Technology (ICT).

Information technology-related contracts means those contracts that include services (including support services), and related resources for information technology.

Organized health care arrangement (see 45 CFR 160.103) means:

- (1) A clinically integrated care setting in which individuals typically receive health care from

more than one health care provider;

(2) An organized system of health care in which more than one covered entity participates and in which the participating covered entities:

(i) Hold themselves out to the public as participating in a joint arrangement; and

(ii) Participate in joint activities that include at least one of the following:

(A) Utilization review, in which health care decisions by participating covered entities are reviewed by other participating covered entities or by a third party on their behalf;

(B) Quality assessment and improvement activities, in which treatment provided by participating covered entities is assessed by other participating covered entities or by a third party on their behalf; or

(C) Payment activities, if the financial risk for delivering health care is shared, in part or in whole, by participating covered entities through the joint arrangement and if protected health information created or received by a covered entity is reviewed by other participating covered entities or by a third party on their behalf for the purpose of administering the sharing of financial risk.

(3) A group health plan and a health insurance issuer or HMO with respect to such group health plan, but only with respect to protected health information created or received by such health insurance issuer or HMO that relates to individuals who are or who have been participants or beneficiaries in such group health plan;

A group health plan and one or more other group health plans each of which are maintained by the same plan sponsor; or

(4) The group health plans described in paragraph (4) of this definition and health insurance issuers or HMOs with respect to such group health plans, but only with respect to protected health information created or received by such health insurance issuers or HMOs that relates to individuals who are or have been participants or beneficiaries in any of such group health plans.

Privacy officer means the HHS official(s) with responsibility for implementing and oversight of privacy related policies and practices that impact a given HHS acquisition.

(b) General. Contractors, subcontractors, their employees, third-parties, and business associates with access to HHS information, information systems, or information technology (IT) or providing and accessing IT-related goods and services, shall adhere to the HHS Cybersecurity Program and the directives and handbooks, complete HHS security training prior to accessing HHS information (including HHS sensitive information and information systems security and privacy) and on an annual basis thereafter, as well as those set forth in the contract specifications, statement of work, or performance work statement. These include, but are not limited to, HHS Personnel Security and Suitability Program, which establishes HHS procedures, responsibilities, and processes for complying with current Federal law, Executive Orders, policies, regulations, standards, and guidance for protecting HHS information, information

systems (see 302.101, Definitions) security and privacy, and adhering to personnel security requirements when accessing HHS information or information systems.

(c) Access to HHS information and HHS information systems.

(1) Contractors are limited in their request for logical or physical access to HHS information or HHS information systems for their employees, subcontractors, third parties and business associates to the extent necessary to perform the services or provide the goods as specified in the contracts, agreements, task, delivery, or purchase orders.

(2) All Contractors, subcontractors, third parties, and business associates working with HHS information are subject to the same investigative requirements as those of HHS appointees or employees who have access to the same types of information. The level and process of background security investigations for Contractors to access HHS information and HHS information systems shall be in accordance with HHS Personnel Security and Suitability Program.

(3) Contractors, subcontractors, third parties, and business associates who require access to national security programs must have a valid security clearance.

(4) The Contractor (and/or any subcontractor) must comply with Executive Order 13556, Controlled Unclassified Information, (implemented at 3 CFR, part 2002) when handling CUI. 32 C.F.R. 2002.4(aa) As implemented the term "handling" refers to "...any use of CUI, including but not limited to marking, safeguarding, transporting, disseminating, re-using, and disposing of the information." 81 Fed. Reg. 63323. The requirements below apply only to nonfederal systems that process, store, or transmit CUI, or that provide security protection for such components. All sensitive information that has been identified as CUI by a regulation or statute, handled by this solicitation/contract, must be –

(i) Marked appropriately;

(ii) Disclosed to authorized personnel on a need-to-know basis;

(iii) Protected in accordance with NIST SP 800-53, Security and Privacy Controls for Information Systems and Organizations applicable baseline if handled by a Contractor system operated on behalf of the agency, or NIST SP 800-171, Protecting Controlled Unclassified Information in Nonfederal Information Systems and Organizations if handled by internal Contractor system; and

(iv) Returned to HHS control, destroyed when no longer needed, or held until otherwise directed. Information and/or data must be disposed of in accordance with NIST SP 800-88, Guidelines for Media Sanitization.

HIPAA business associate agreements. Under the HIPAA Privacy and Security Rules (see 45 CFR 164), pursuant to 45 CFR 164.502(e)(1), a covered entity may disclose protected health information to a business associate and may allow a business associate to create, receive, maintain, or transmit protected health information on its behalf, if the covered entity obtains satisfactory assurance that the business associate will appropriately safeguard the information. A covered entity is not required to obtain such satisfactory assurances

from a business associate that is a subcontractor of a covered entity's business associate. Additionally, a business associate may disclose protected health information to a business associate that is a subcontractor and may allow the subcontractor to create, receive, maintain, or transmit protected health information on its behalf, if the business associate obtains satisfactory assurances, in accordance with 45 CFR 164.504(e)(1)(i), that the subcontractor will appropriately safeguard the information. The satisfactory assurances required by 45 CFR 164.504(e)(1) of this section shall be documented through a written contract or other written agreement or arrangement with the business associate that meets the applicable requirements of 45 CFR 164.504(e). The contracts shall also include breach reporting policies and procedures for suspected or confirmed breaches of protected health information. The contract shall impose a duty to cooperate with the healthcare component and/or HHS breach investigation and response and must require all subcontractors to comply with the same HIPAA Rules requirements as a condition of receiving government data.

(i) Contractors or entities required to execute business associate agreements for contracts and other agreements become HHS business associates. Business associate agreements are issued by HHS or may be issued by other HHS programs in support of HHS. The HIPAA Privacy Rule requires HHS to execute compliant business associate agreements with persons or entities that create, receive, maintain, or transmit HHS PHI or that will store, generate, access, exchange, process, or utilize such PHI in order to perform certain activities, functions, or services to, for, or on behalf of HHS. There may be other HHS components or staff offices which also provide certain services and support to HHS and must receive PHI in order to do so. If these components award contracts or enter into other agreements, purchase/delivery orders, modifications, and issue governmentwide purchase card transactions to help in the delivery of these services to HHS, they will also fall within the requirement to obtain a satisfactory assurance from these contractors by executing a business associate agreements.

(ii) Business associate agreement flow down to subcontractors. A prime contractor required to execute a business associate agreement shall also obtain a satisfactory assurance, in the form of a business associate agreement, of its subcontractors who will also create, receive, maintain, or transmit PHI or that will store, generate, access, exchange, process, or utilize such PHI will comply with HIPAA Rules requirements to the same degree as the Contractor. A contractor employing a subcontractor who creates, receives, maintains, or transmits PHI or that will store, generate, access, exchange, process, or utilize such PHI under a contract or agreement is required to execute a business associate agreement with each of its subcontractors which also obligates the subcontractor (i.e., also a business associate) to provide the same protections and safeguards and agree to the same disclosure restrictions to PHI that is required of the covered entity and the prime contractor.

(d) Contractor operations required to be in United States. Custom software development and outsourced operations must be located in the U.S. to the maximum extent practicable. If such services are proposed to be performed outside the continental United States, and are not otherwise disallowed by other Federal law, regulations or policy, or other HHS policy or other mandates as stated in the contract, specifications, statement of work or performance work statement (including applicable business associate agreements), the Contractor/subcontractor must state in its proposal where all non-U.S. services are provided. At a minimum, the Contractor/subcontractor must include a detailed Information System Security Plan, for review and approval by the Contracting Officer, specifically to address mitigation of the resulting problems of communication, control, and data protection.

(e) Roster of employees. Contractors and subcontractors shall provide a roster containing the name, position, e-mail address, phone number, and responsibilities of each employee, including subcontractors, performing work under the contract to develop, have the ability to access, or host and/or maintain a government information system(s). The roster must be submitted to Contracting Officer within _____ [Contracting Officer to insert the number of days] days stated number of days from the effective date of the contract. Revisions to the roster as a result of staffing changes must be submitted within the number of days of the change provided by the Contracting Officer. The Contracting Officer, or the Contracting Officer's Representative (COR), will notify the Contractor of the appropriate level of investigation required for each staff member based on the information provided on the roster. If

an employee is filling a new position, the Contractor must provide a position description and the Government will determine the appropriate suitability level.

(f) Contractor/subcontractor employee reassignment and termination notification. Contractors and subcontractors shall provide written notification to the Contracting Officer and COR immediately, and not later than four (4) hours, when an employee working on an HHS information system or with access to HHS information is reassigned or leaves the Contractor or subcontractor's employment on the cognizant HHS contract. The Contracting Officer and COR must also be notified immediately by the Contractor or subcontractor prior to an unfriendly termination.

(g) Non-disclosure agreement. The Contractor and subcontractors shall submit completed non-disclosure agreements, as provided by the Contracting Officer, for each employee having access to non-public government information under this contract. The non-disclosure agreements shall be submitted to the Contracting Officer prior to the performance of work.

(h) HHS information custodial requirements. (1) Release, publication, and use of data. Information made available to a Contractor or subcontractor by HHS for the performance or administration of a contract or information developed by the Contractor/subcontractor in performance or administration of a contract shall be used only for the stated contract purpose and shall not be used in any other way without HHS prior written approval. This clause expressly limits the Contractor's/subcontractor's rights to use data as described in 52.227-14, Rights in Data—General, paragraph (d).

(2) Media sanitization. HHS information shall not be co-mingled with any other data on the Contractor/subcontractor's information systems or media storage systems in order to ensure federal and HHS requirements related to data protection, information segregation, classification requirements, and media sanitization can be met (see HHS Cybersecurity Program). HHS reserves the right to conduct scheduled or unscheduled on-site inspections, assessments, or audits of Contractor and subcontractor IT resources, information systems and assets to ensure data security and privacy controls, separation of data and job duties, and destruction/media sanitization procedures are in compliance with Federal and HHS requirements. The Contractor and subcontractor will provide all necessary access and support to HHS and/or GAO staff during periodic control assessments or audits.

(3) Data retention, destruction, and contractor self-certification. The Contractor and its subcontractors are responsible for collecting and destroying any HHS data provided, created, or

stored under the terms of this contract, to a point where HHS data or materials are no longer readable or reconstructable to any degree, in accordance with NIST SP 800-88, Guidelines for Media Sanitization, or subsequent directive. Prior to termination or completion of this contract, the Contractor/subcontractor must provide its plan for destruction or return of all HHS data in its possession accordance with contract requirements or Contracting Officer instructions for disposition, including compliance with National Institute of Standards and Technology (NIST) SP 800-88, Guidelines for Media Sanitization, for the purposes of media sanitization on all IT equipment. The Contractor must certify in writing to the Contracting Officer within 30 days of termination of the contract that the data destruction requirements in this paragraph have been met.

(4) Return of HHS data and information. When information, data, documentary material, records and/or equipment is no longer required, it shall be returned to the HHS (as stipulated by the Contracting Officer or the COR) or the Contractor/subcontractor must hold it until otherwise directed. Items returned will be hand carried, securely mailed, emailed, or securely electronically transmitted to the Contracting Officer or to the address as provided in the contract or by the assigned COR, and/or accompanying business associate agreement. Depending on the method of return, Contractor/subcontractor must store, transport, or transmit HHS sensitive information, when permitted by the contract using HHS-approved encryption tools that are, at a minimum, validated under Federal Information Processing Standards (FIPS) 140-3 (or its successor). If mailed, Contractor/subcontractor must send via a trackable method (USPS, UPS, Federal Express, etc.) and immediately provide the Contracting Officer with the tracking information. No information, data, documentary material, records or equipment will be destroyed unless done in accordance with the terms of this contract and the HHS Agency Records Control Schedules (2019).

(5) Use of HHS data and information. The Contractor/subcontractor must receive, gather, store, back up, maintain, use, disclose and dispose of HHS information only in compliance with the terms of the contract and applicable Federal and HHS information confidentiality and security laws, regulations, and policies. If Federal or HHS information confidentiality and security laws, regulations, and policies become applicable to the HHS information or information systems after execution of the contract, or if the NIST issues or updates applicable FIPS or Special Publications (SP) after execution of this contract, the parties agree to negotiate in good faith to implement the information confidentiality and security laws, regulations and policies for this contract as a result of any updates, if required.

(6) Copying HHS data or information. The Contractor/subcontractor shall not make copies of HHS information except as authorized and necessary to perform the terms of the contract or to preserve electronic information stored on Contractor/subcontractor electronic storage media for restoration in case any electronic equipment or data used by the Contractor/subcontractor needs to be restored to an operating state. If copies are made for restoration purposes, after the restoration is complete, the copies must be appropriately destroyed.

(7) Violation of information custodial requirements. If HHS determines that the Contractor has violated any of HHS information confidentiality, privacy, or security provisions, it shall be sufficient grounds for HHS to withhold payment to the Contractor or third-party or terminate the contract for default in accordance with FAR part 49 or terminate for cause in accordance with FAR 12.403.

(8) Encryption. The Contractor/subcontractor must store, transport, or transmit HHS sensitive information, when permitted by the contract, using cryptography, HHS encryption policies, and HHS-approved encryption tools that are, at a minimum, validated under FIPS 140-3 (or its successor).

(9) Firewall and web services security controls. The Contractor/subcontractor's firewall and Web services security controls, if applicable, shall meet or exceed HHS minimum requirements. HHS Configuration Standards Guidelines are available upon request.

(10) Disclosure of HHS data and information. Except for uses and disclosures of HHS information authorized in a cognizant contract for performance of the contract, the Contractor/subcontractor may use and disclose HHS information only in two other situations: (i) subject to paragraph 10 of this section, in response to a court order from a court of competent jurisdiction, or (ii) with HHS prior written approval. The Contractor/subcontractor must refer all requests for, demands for production of, or inquiries about, HHS information and information systems to the Contracting Officer for response. If the Contractor/subcontractor is in receipt of a court order or other request or believes it has a legal requirement to disclose HHS information, that Contractor/subcontractor shall immediately refer such court order or other request to the Contracting Officer for response. If the Contractor or subcontractor discloses information on behalf of HHS, the Contractor and/or subcontractor must maintain an accounting of disclosures. Accounting of Disclosures documentation maintained by the Contractor/subcontractor will include the name of the individual to whom the information pertains, the date of each disclosure, the nature or description of the information disclosed, a brief statement of the purpose of each disclosure or, in lieu of such statement, a copy of a written request for a disclosure, and the name and address of the person or agency to whom the disclosure was made. The Contractor/subcontractor will provide its Accounting of Disclosures upon request and within 15 calendar days to the assigned COR and Privacy Officer. Accounting of disclosures should be provided electronically via encrypted email to the COR and designated HHS facility Privacy Officer as provided in the contract, business associate agreement, or by the Contracting Officer. If providing the Accounting of disclosures electronically cannot be done securely, the Contractor/subcontractor will provide copies via trackable methods (UPS, USPS, Federal Express, etc.) immediately, providing the designated COR and Privacy Officer with the tracking information.

(11) Compliance with privacy statutes and applicable regulations. The Contractor/subcontractor shall not disclose HHS information protected by any of HHS privacy statutes or applicable regulations including, but not limited to, the Privacy Act of 1974 or the HIPAA Rules. If the Contractor/subcontractor is in receipt of a court order or other requests for HHS information or has questions if it can disclose information protected under the above-mentioned confidentiality statutes because it is required by law, that Contractor/subcontractor shall immediately refer such court order or other request to the Contracting Officer for response.

(i) Compliance with identification policies. Contractors shall comply with the Homeland Security Presidential Directive (HSPD)-12, Policy for a Common Identification Standard for Federal Employees and Contractors; OMB M-05-24; OMB M-19-17; FIPS 201, Personal Identity Verification (PIV) of Federal Employees and Contractors; HHS HSPD-12 policy; HHS Policy for Information Security and Privacy Protection (IS2P) Control Catalog, and Executive Order 13467, Part 1, section 1.2.

(j) Report of known or suspected incident or breach. The Contractor, subcontractor, third-party affiliate or business associate, and its employees shall notify HHS immediately via the Contracting Officer and the COR or within one (1) hour of a known or suspected incident or breach. The initial notification may first be made verbally but must be followed up in writing within one (1) hour. Report all actual or suspected incident and breach information to the Contracting Officer and the COR as identified in the contract or as directed in the contract, within one hour of discovery or suspicion.

(1) Such issues shall be remediated as quickly as is practical, but in no event longer than _____ days [Fill in: Contracting Officer fills in the number of days]. The Contractor shall notify the Contracting Officer in writing.

(2) When the security fixes involve installing third party patched (e.g., Microsoft OS patches or Adobe Acrobat), the Contractor will provide written notice to HHS that the patch has been validated as not affecting the systems within 10 working days. When the Contractor is responsible for operations or maintenance of the systems, they shall apply the security fixes within ____ [Fill in: Contracting Officer fills in the number of days in consultation with requiring activity].

(3) All other vulnerabilities shall be remediated in a timely manner based on risk, in accordance with the timelines specified in the HHS Policy for Vulnerability Management, and the HHS Standard for Plan of Action and Milestones (POAM) Management and Reporting. Contractors shall notify the Contracting Officer, and COR within 2 business days after remediation of the identified vulnerability. Exceptions to this paragraph (e.g., for the convenience of HHS) must be requested by the Contractor through the COR and shall only be granted with approval of the Contracting Officer and the Office of the Chief Information Officer (OCIO). These exceptions will be tracked by the Contractor in concert with the Government in accordance with HHS Policy for IT Procurements–Security and Privacy Language.

(k) Incident and breach investigation. (1) The Contractor/ subcontractor shall immediately notify the Contracting Officer and COR for the contract of any known or suspected incident or breach (see definitions, paragraph (a)), or any other unauthorized disclosure of sensitive information, including that contained in system(s) to which the Contractor/subcontractor has access.

(2) To the extent known by the Contractor/subcontractor, the Contractor/ subcontractor's notice to HHS shall identify the information involved, an estimate of the number of potentially impacted individuals, the circumstances surrounding the incident (including to whom, how, when, and where the HHS information or assets were placed at risk or compromised), and any other information that the Contractor/subcontractor considers relevant.

(3) With respect to unsecured protected health information, the business associate is deemed to have discovered an incident as defined above when the business associate either knew, or by exercising reasonable diligence should have been known to an employee of the business associate. Upon discovery, the business associate must notify HHS of the incident immediately within one hour of discovery or suspicion as agreed to in the business associate agreement.

(4) In instances of theft or break-in or other criminal activity, the Contractor/subcontractor must concurrently report the incident to the appropriate law enforcement entity (or entities) of jurisdiction. The Contractor, its employees, and its subcontractors and their employees shall cooperate with HHS and any law enforcement authority responsible for the investigation and prosecution of any possible criminal law violation(s) associated with any incident. The Contractor/subcontractor shall cooperate with HHS in any civil litigation to recover HHS information, obtain monetary or other compensation from a third party for damages arising from any incident, or obtain injunctive relief against any third party arising from, or related to, the incident.

(l) Incident and breach notification requirements. (1) The Contractor/subcontractor shall provide notice to HHS of an incident as set forth in the incident and breach investigation section of this clause. The Contractor shall fully cooperate with HHS or third-party entity performing an independent risk analysis on behalf of HHS. Failure to cooperate may be deemed a material incident or breach and grounds for contract termination.

(2) The Contractor/subcontractor shall fully cooperate with the HHS Computer Security Incident Response Center (CSIRC), HHS Breach Response Team, Operating Divisions (OPDIVs), Staff Divisions (STAFFDIVs), other stakeholders or any Government agency conducting an analysis regarding any notice of an incident or breach, potential incident or breach, or incident which may require the Contractor to provide information to the Government or third-party performing a risk analysis for HHS, and shall address all relevant information concerning the incident or breach, including the following:

(i) Nature of the event (loss, theft, unauthorized access).

(ii) Description of the event, including:

(A) Date of occurrence.

(B) Date of incident or breach detection.

(C) Data elements involved, including any PII, such as full name, social security number, date of birth, home address, account number, disability code.

(D) Number of individuals affected or potentially affected.

(E) Names of individuals or groups affected or potentially affected.

(F) Ease of logical data access to the lost, stolen or improperly accessed data in light of the degree of protection for the data, e.g., unencrypted, plain text.

(G) Amount of time the data has been out of HHS control.

(H) The likelihood that the sensitive information will or has been compromised (made accessible to and usable by unauthorized persons).

- (I) Known misuses of data containing sensitive information, if any.
- (J) Assessment of the potential harm to the affected individuals.
- (K) Incident or breach analysis as outlined in the HHS Breach Response Policy and Plan, as appropriate.
- (L) Whether credit protection services may assist record subjects in avoiding or mitigating the results of identity theft based on the sensitive information that may have been compromised.
- (M) Steps taken in response to mitigate or prevent a repetition of the incident.

(m) Training. (1) All Contractor employees and subcontractor employees requiring access to HHS information or HHS information systems shall complete the following before being granted access to HHS information and its systems:

- (i) On an annual basis, successfully complete the HHS Privacy and Information Security Awareness and HHS Information Security Rules of Behavior training.
- (ii) On an annual basis, sign and acknowledge (either manually or electronically) understanding of and responsibilities for compliance with the HHS Information Security Rules of Behavior, relating to access to HHS information and information systems.
- (iii) Successfully complete any additional cyber security or privacy training, as required for HHS personnel with equivalent information system access.

(2) The Contractor shall provide to the Contracting Officer and/or the COR a copy of the training certificates and affirmation that HHS Information Security Rules of Behavior signed by each applicable employee have been completed and submitted within five (5) days of the initiation of the contract and annually thereafter, as required.

(3) Failure to complete the mandatory annual training and acknowledgement of the HHS Information Security Rules of Behavior, within the timeframe required, is grounds for suspension or termination of all physical or electronic access privileges and removal from work on the contract until such time as the training and documents are complete.

(n) Subcontract flow down. The Contractor shall include the substance of this clause, including this paragraph (k), in subcontracts, third-party agreements, and business associate agreements, of any amount and in which subcontractor employees, third-party servicers/employees, and business associates will perform functions where they will have access to HHS information (including HHS sensitive information), information systems, information technology (IT) or providing and accessing information technology-related contract services, support services, and related resources (see HHSAR 302.101 definition of information technology-related contracts.)

(End of clause)

HHSAR 352.232-71 Electronic Submission of Payment Requests (Feb 2022)

(a) *Definitions.* As used in this clause—

(1) “*Payment request*” means a bill, voucher, invoice, or request for contract financing payment with associated supporting documentation. The payment request must comply with the requirements identified in FAR 32.905(b), “Content of Invoices” and the applicable Payment clause included in this contract.

(b) Except as provided in paragraph (c) of this clause, the Contractor shall submit payment requests electronically using the Department of Treasury Invoice Processing Platform (IPP) or successor system. Information regarding IPP, including IPP Customer Support contact information, is available at www.ipp.gov or any successor site.

(c) The Contractor may submit payment requests using other than IPP only when the Contracting Officer authorizes alternate procedures in writing in accordance with HHS procedures.

(d) If alternate payment procedures are authorized, the Contractor shall include a copy of the Contracting Officer's written authorization with each payment request.

(End of Clause)

HHSAR 352.239-71 Security Requirements for Information Technology Resources. (Deviation)(Feb 2024)

(a) *Definitions.* As used in this clause—

Information technology has the same meaning in FAR 2.101.

Information and communication technology (ICT) also means information technology (see FAR 2.101 for definitions).

Information system security plan means a formal document that provides an overview of the security requirements for an information system or an information security program and describes the security controls in place or planned for meeting those requirements.

(b) *Responsibilities.* The Contractor shall be responsible for information technology security for all systems connected to a Department of Health and Human Services (HHS) network or operated by the Contractor for HHS, regardless of location. This clause is applicable to all or any part of the contract that includes information technology resources or services in which the Contractor has physical or other system access to HHS information that directly supports the mission of HHS. Examples of tasks that require security provisions include—

(1) Hosting of HHS e-Government sites or other information technology operations;

(2) Acquisition, transmission, or analysis of data owned by HHS with significant replacement cost should the contractor's copy be corrupted; and

(3) Access to HHS general support systems/major applications at a level beyond that granted the general public, e.g., bypassing a firewall.

(c) Information system security plan. The Contractor shall develop, provide, implement, and maintain an Information System Security Plan. HHS information system and platform information technology systems must have a security plan that provides an overview of the security requirements for the system and describes the security controls in place or the plan for meeting those requirements. Generally, this plan shall describe the processes and procedures that the Contractor will follow to ensure appropriate security of information system resources developed, processed, or used under this contract. The security plan should include implementation status, responsible entities, resources, and estimated completion dates. Security plans may also include, but are not limited to, a compiled list of system characteristics or qualities required for system registration, and key security-related documents such as a risk assessment, privacy impact assessment (PIA), system interconnection agreements, contingency plan, security configurations, configuration management plan, incident response plan, and the system privacy plan, as determined by the Contracting Officer's Representative. The plan shall address the specific contract requirements regarding information systems or related support or services included in the contract, to include the PWS or SOW. The Contractor's Information System Security Plan shall comply with applicable Federal Laws that include, but are not limited to, 40 U.S.C. 11331, the Federal Information Security Modernization Act (FISMA) of 2014 and the E-Government Act of 2002. The plan shall meet information technology security requirements in accordance with Federal and HHS policies and procedures, and as amended during the term of this contract, and include, but are not limited to the following:

(1) OMB Circular A-130, Managing Information as a Strategic Resource;

(2) National Institute of Standards and Technology (NIST) Guidelines;

(3) Federal Information Processing Standard (FIPS) 200; and

(4) HHS Cybersecurity Program related to HHS information (including HHS sensitive information and information systems security and privacy), as well as those set forth in the contract specifications, statement of work, or performance work statement. These include, but are not limited to, the Cyber Security Checklist and Cyber Security Infographic at <https://www.hhs.gov/hipaa/for-professionals/security/guidance/cybersecurity/index.html>, which provides HHS procedures, responsibilities, and processes for complying with current Federal law, Executive Orders, policies, regulations, standards and guidance for protecting HHS information, information systems (see 302.101, Definitions), information technology, and ICT, security and privacy, and adhering to personnel security requirements when accessing HHS information or information systems.

(d) Submittal of plan. Within 60 days after contract award, the Contractor shall submit the Information System Security Plan to the Contracting Officer for review and approval.

(e) Authority to Operate (ATO). As required by current HHS policy, the Contractor shall submit written proof of information technology security accreditation with a valid ATO to the Contracting Officer. Such written proof may be furnished either by the Contractor or by a third

party. Accreditation shall be in accordance with HHS policy available from the Contracting Officer upon request. The Contractor shall submit for acceptance by the Contracting Officer along with this ATO a final security plan, risk assessment, security test and evaluation, privacy threshold analysis or privacy impact assessment, and a disaster recovery plan/continuity of operations plan.

Annual validation. On an annual basis, the Contractor shall verify in writing to the Contracting Officer that the IT Security Plan remains valid.

(f) Banners. The Contractor shall ensure that the official HHS banners are displayed on all HHS systems (both public and private) operated by the Contractor that contain Privacy Act or other sensitive information before allowing anyone access to the system. The Office of Information Technology will make official HHS banners available to the Contractor.

(g) Screening and access. The Contractor shall screen all personnel requiring privileged access or limited privileged access to systems operated by the Contractor for HHS or interconnected to an HHS network in accordance with HHS policies referenced in paragraph (c).

(h) Training. The Contractor shall ensure that its employees performing services under this contract complete HHS security awareness and privacy training on an annual basis. This includes signing an acknowledgment on an annual basis that they have read, understand, and agree to abide by the HHS Rules of Behavior for the Use of HHS Information and IT Resources Policy as required; FAR 39.105, Privacy; clause 352.204-71, Information and Information Systems Security, and this clause.

(i) Government access. The Contractor shall provide the Government access to the Contractor's and subcontractors' facilities, installations, operations, documentation, information systems, databases, and personnel used in performance of the contract. The Contractor shall provide access to enable a program of information technology inspection (to include vulnerability testing), investigation and audit (to safeguard against threats and hazards to the integrity, availability, and confidentiality of HHS data or to the function of information technology systems operated on behalf of HHS), and to preserve evidence of computer crime.

(j) Notification of termination of employees. The Contractor shall immediately notify the Contracting Officer when an employee who has access to HHS information systems or data terminates employment.

(k) Subcontract flow down requirement. The Contractor shall incorporate and flow down the substance of this clause to all subcontracts that meet the conditions in paragraph (a) of this clause.

(End of clause)

CDC0_G018 Payment by Electronic Funds Transfer (Feb 2018)

(a) The Government shall use electronic funds transfer to the maximum extent possible when making payments under this contract. FAR 52.232-33, Payment by Electronic

Funds Transfer –System for Award Management, in Section I, requires the contractor to designate in writing a financial institution for receipt of electronic funds transfer payments.

(b) In the case that EFT information is not within the System of Award Management, FAR 52.232-34 requires mandatory submission of Contractor’s EFT information directly to the office designated in this contract to receive that information (hereafter: “designated office”); see below. The contractor shall submit the EFT information within the form titled “ACH Vendor/Miscellaneous Payment Enrollment Form” to the address indicated below. Note: The form is either attached to this contract (see Section J, List of Attachments) or may be obtained by contacting the Contracting Officer or the CDC Office of Financial Resources at 678-475-4510.

(c) In cases where the contractor has previously provided such information, i.e., pursuant to a prior contract/order, and been enrolled in the program, the form is not required unless the designated financial institution has changed.

(d) The completed form shall be mailed after award, but no later than 14 calendar days before an invoice is submitted, to the following address:

The Centers for Disease Control and Prevention Office of Financial Resources (OFR)
P.O. Box 15580 Atlanta, GA 30333
Or – Fax copy to: 404-638-5342

CDCA_H042 Records Management Obligations (Jun 2020)

A. Applicability

The following applies to all Contractors whose employees create, work with, or otherwise handle Federal records, as defined in Section B, regardless of the medium in which the record exists.

B. Definitions

“Federal record” as defined in 44 U.S.C. § 3301, includes all recorded information, regardless of form or characteristics, made or received by a Federal agency under Federal law or in connection with the transaction of public business and preserved or appropriate for preservation by that agency or its legitimate successor as evidence of the organization, functions, policies, decisions, procedures, operations, or other activities of the United States Government or because of the informational value of data in them.

The term Federal record:

1. includes Centers for Disease Control and Prevention (CDC) records.
2. does not include personal materials.
3. applies to records created, received, or maintained by Contractors pursuant to their CDC contract.
4. may include deliverables and documentation associated with deliverables.

C. Requirements

1. Contractor shall comply with all applicable records management laws and regulations, as well as National Archives and Records Administration (NARA) records policies, including but not limited to the Federal Records Act (44 U.S.C. chs. 21, 29, 31, 33), NARA regulations at 36 CFR Chapter XII Subchapter B, and those policies associated with the safeguarding of records covered by the Privacy Act of 1974 (5 U.S.C. 552a). These policies include the preservation of all records, regardless of form or characteristics, mode of transmission, or state of completion.
2. In accordance with 36 CFR 1222.32, all data created for Government use and delivered to, or falling under the legal control of, the Government are Federal records subject to the provisions of 44 U.S.C. chapters 21, 29, 31, and 33, the Freedom of Information Act (FOIA) (5 U.S.C. 552), as amended, and the Privacy Act of 1974 (5 U.S.C. 552a), as amended and must be managed and scheduled for disposition only as permitted by statute or regulation.
3. In accordance with 36 CFR 1222.32, Contractor shall maintain all records created for Government use or created in the course of performing the contract and/or delivered to, or under the legal control of the Government and must be managed in accordance with Federal law. Electronic records and associated metadata must be accompanied by sufficient technical documentation to permit understanding and use of the records and data.
4. CDC and its contractors are responsible for preventing the alienation or unauthorized destruction of records, including all forms of mutilation. Records may not be removed from the legal custody of CDC or destroyed except for in accordance with the provisions of the agency records schedules and with the written concurrence of the Head of the Contracting Activity. Willful and unlawful destruction, damage or alienation of Federal records is subject to the fines and penalties imposed by 18 U.S.C. 2701. In the event of any unlawful or accidental removal, defacing, alteration, or destruction of records, Contractor must report to the Contracting Officer and the Contracting Officer's Representative. The agency must report promptly to NARA in accordance with 36 CFR 1230.
5. The Contractor shall immediately notify the appropriate Contracting Officer upon discovery of any inadvertent or unauthorized disclosures of information, data, documentary materials, records, or equipment. Disclosure of non-public information is limited to authorized personnel with a need-to-know as described in the contract. The Contractor shall ensure that the appropriate personnel, administrative, technical, and physical safeguards are established to ensure the security and confidentiality of this information, data, documentary material, records and/or equipment is properly protected. The Contractor shall not remove material from Government facilities or systems, or facilities or systems operated or maintained on the Government's behalf, without the express written permission of the Head of the Contracting Activity. When information, data, documentary material, records and/or equipment is no longer required, it shall be returned to CDC control or the Contractor must hold it until otherwise directed. Items returned to the Government shall be hand carried, mailed, emailed, or securely electronically transmitted to the Contracting Officer or address prescribed in the contract. Destruction of records is EXPRESSLY PROHIBITED unless in accordance with Paragraph (4).

6. The Contractor is required to obtain the Contracting Officer's approval prior to engaging in any contractual relationship (sub-contractor) in support of this contract requiring the disclosure of information, documentary material and/or records generated under, or relating to, contracts. The Contractor (and any sub-contractor) is required to abide by Government and CDC guidance for protecting sensitive, proprietary information, classified, and controlled unclassified information.
7. The Contractor shall only use Government IT equipment for purposes specifically tied to or authorized by the contract and in accordance with CDC policy.
8. The Contractor shall not create or maintain any records containing any non-public CDC information that are not specifically tied to or authorized by the contract.
9. The Contractor shall not retain, use, sell, or disseminate copies of any deliverable that contains information covered by the Privacy Act of 1974 or that which is generally protected from public disclosure by an exemption to the Freedom of Information Act.
10. Training. All Contractor employees assigned to this contract who create, work with, or otherwise handle records are required to take CDC-provided records management training. The Contractor is responsible for confirming training has been completed according to agency policies, including initial training and any annual or refresher training.

D. Flowdown of requirements to subcontractors

1. The Contractor shall incorporate the entire substance of the terms and conditions herein, including this paragraph, in all subcontracts under this contract, and must require written subcontractor acknowledgment of same.
2. Violation by a subcontractor of any provision set forth herein will be attributed to the Contractor.

CDCP_G009 Contracting Officer (Jul 1999)

- a. The Contracting Officer is the only individual who can legally commit the Government to the expenditure of public funds. No person other than the Contracting Officer can make any changes to the terms, conditions, general provisions, or other stipulations of this contract.
- b. No information, other than that which may be contained in an authorized modification to this contract, duly issued by the Contracting Officer, which may be received from any person employed by the United States Government, or otherwise, shall be considered grounds for deviation from any stipulation of this contract.

(End of Clause)

CDC0_G008 Contracting Officer's Representative (COR) (Jul 2017)

Performance of the work hereunder shall be subject to the technical directions of the designated COR for this contract.

As used herein, technical directions are directions to the Contractor which fill in details, suggests possible lines of inquiry, or otherwise completes the general scope of work set forth herein. These technical directions must be within the general scope of work and may not alter the scope of work or cause changes of such a nature as to justify an adjustment in the stated contract price/cost, or any stated limitation thereof.

In the event that the Contractor believes full implementation of any of these directions may exceed the scope of the contract, he or she shall notify the originator of the technical direction and the Contracting Officer, immediately or as soon as possible, in a letter or e-mail separate of any required report(s). No technical direction, nor its fulfillment, shall alter or abrogate the rights and obligations fixed in this contract.

The Government COR is not authorized to change any of the terms and conditions of this contract. Contract changes shall be made only by the Contracting Officer by properly written modification(s) to the contract.

The Government will provide the Contractor with a copy of the COR

delegation memorandum upon request. (End of Clause)

CDC0_H049 Non-Disclosure Agreement for Contractor and Contractor Employees (Jun 2020)

(a) The contractor and contractor employees shall prepare and submit Non-Disclosure Agreements (NDA) to the Contracting Officer prior to access of government information or the commencement of work at CDC.

(b) The NDAs, at Exhibit I and II, are required in service contracts where contractor's employees will have access to non-public and procurement-sensitive information while performing functions in support of the Government. The NDA also requires contractor's employees properly identify themselves as employees of a contractor when communicating or interacting with CDC employees, employees of other governmental entities, and members of the public (when communication or interaction relates to the contractor's work with the CDC). The Federal Acquisition Regulation (FAR) 37.114 (c), states "All contractor personnel attending meetings, answering Government telephones, and working in other situations where their contractor status is not obvious to third parties are required to identify themselves as such to avoid creating an impression in the minds of members of the public or Congress that they are Government officials, unless, in the judgment of the agency, no harm can come from failing to identify

themselves. They must also ensure that all documents or reports produced by contractors are suitably marked as contractor products or that contractor participation is appropriately disclosed.”

(c) The contractor shall inform contractor employees of the identification requirements by which they must abide and monitor employee compliance with the identification requirements.

(d) During the contract performance period, the contractor is responsible to ensure that all additional or replacement contractors’ employees sign an NDA, and it is submitted to the Contracting Officer prior to commencement of their work with the CDC.

(e) Contractor employees in designated positions or functions that have not signed the appropriate NDA shall not have access to any non-public, procurement sensitive information or participate in government meetings where sensitive information may be discussed.

(f) The Contractor shall prepare and maintain a current list of employees working under NDAs and submit to the Contracting Officer upon request during the contract period of performance. The list should at a minimum include: contract number, employee’s name, position, date of hire and NDA requirement.